THE MATERNITY METHOD

NCLEX CHEAT SHEET

Labor Emergencies

Maternal & Newborn › Intrapartum Complications

The Big Picture Labor emergencies happen **fast**. The nurse's job: **recognize it, call for help, start the first actions**, and **get ready for delivery**. Stay with the patient, keep her informed, and **document times**.

KNOW THE NUMBERS

Right after ROM	McRoberts	Never	< 3 hrs	Sudden brady
Check FHR: **cord prolapse**?	**First** move for shoulder dystocia	**Fundal pressure** in dystocia	**Precipitous** labor	Often the **first** sign of uterine rupture

Prolapsed Umbilical Cord

The cord slips **ahead of baby** and gets squeezed → baby loses oxygen. **Risks:** rupture before head is engaged, breech, extra fluid, small or preterm baby, twins.

- **Signs:** cord seen or felt; sudden **variable decels** or **bradycardia** after membranes rupture
- **Gloved hand in the vagina: lift the presenting part OFF the cord** and keep it there
- **Call for help**; do **not** push the cord back in
- **Knee-chest** or **Trendelenburg** position
- Cord outside? Cover with **sterile saline-soaked** gauze
- Stop oxytocin, O_2 per protocol, IV fluids → **emergency cesarean**

Shoulder Dystocia

Head is out but the **front shoulder is stuck** behind the pubic bone. **Risks:** big baby, **diabetes**, obesity, past dystocia, long 2nd stage, vacuum/forceps.

- **Sign: "turtle sign"**: the head **pulls back** against the perineum
- **Call for help**, note the **time**
- **McRoberts**: pull mom's **thighs back toward her belly**
- **Suprapubic pressure** (just above the pubic bone), **never fundal pressure**
- **Newborn:** check for **broken clavicle** and **Erb palsy** (arm doesn't move, uneven Moro)

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Uterine Rupture

- **Risks:** prior cesarean scar (especially classical/vertical), TOLAC, too much oxytocin, trauma
- **Signs:** sudden **FHR changes/bradycardia**, sharp "tearing" pain, contractions **stop**, baby moves **back up** (loss of station), **shock**
- **Act:** call for help, IV fluids, blood, **emergency cesarean**

Amniotic Fluid Embolism

- Fluid enters mom's blood → **sudden collapse** in labor or right after birth
- **Signs:** sudden **trouble breathing**, low O_2, **low BP**, cardiac arrest, then **DIC bleeding**
- **Act:** rapid response/code, **CPR** with **left uterine displacement**, O_2, blood products, fast delivery

Precipitous Birth (Baby Coming Fast)

- **Stay with her** and call for help; don't leave to get supplies
- Support the head; let it deliver **slowly between contractions**; **don't pull**
- Check for **cord around the neck**; slip it over the head if loose
- **Dry, warm**, skin-to-skin; watch mom for bleeding and tears

MEMORY HACK · WHO GOES WHERE

Match the emergency to the first move

Cord prolapse	Dystocia	Rupture	Embolism	Precipitous
lift baby off the cord	**McRoberts** + suprapubic	**surgery** + fluids	**CPR** + O_2	**stay** + support the head

RED FLAGS · STOP AND CALL THE PROVIDER

Bradycardia or deep decels after membranes rupture · **Turtle sign** · **Sudden severe pain** with a scarred uterus · Sudden **shortness of breath** or collapse · **Oozing** from IV sites (DIC)

MUST-KNOW FOR THE NCLEX

- **Cord prolapse:** hand in the vagina lifting the head off the cord + knee-chest. Stay until cesarean.
- **Shoulder dystocia:** McRoberts + **suprapubic** pressure. **Never fundal** pressure.
- **Uterine rupture:** suspect with a prior cesarean + sudden FHR changes + loss of station.
- **First nursing action** in almost every emergency: **call for help** and stay with the patient.

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QUICK CHECK · TEST YOURSELF

1. After seeing the cord at the vaginal opening, the nurse first _____.
2. The first maneuver for shoulder dystocia is _____.
3. Pressure applied during shoulder dystocia goes _____ the pubic bone, never on the fundus.
4. A baby born after shoulder dystocia should be checked for a broken _____.
5. Sudden collapse and DIC right after birth suggests _____.

Answers: 1. lifts the presenting part off the cord with a gloved hand · 2. McRoberts · 3. just above (suprapubic) · 4. clavicle · 5. amniotic fluid embolism

For study use only. Always follow facility protocols, current guidelines, and provider orders.